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Case Number: 25NNCV03515 Hearing Date: July 30, 2026 Dept: X

#17 - ELIZABETH MONTOYA vs KHALED TAWANSY, M.D., et al.

ORDER TO SHOW CAUSE RE: RESPONSIVE PLEADING / DEFAULT ON FAC

A responsive pleading remains outstanding as to Defendant Sunalp. The Order to Show Cause hearing is continued to October 29, 2026 at 8:30 a.m. in Department X.

DEMURRER

AND MOTION TO STRIKE FIRST AMENDED COMPLAINT

Plaintiff Elizabeth Montoya filed this medical malpractice action on May 22, 2025 against Defendants Khaled Tawansy, M.D., Raymond Renaissance Surgery Center, LLC ("RRSC"), Golden State Eye Medical Group, Inc., and Sunalp Laser Vision, Inc.

On February 20, 2026, the Court sustained RRSC's demurrer to the original complaint as to the claims for lack of informed consent, fraud/concealment/deceit, and Health and Safety Code section 123110 causes of action with leave to amend. Plaintiff filed the FAC on May 13, 2026.

RRSC demurs to the

First Amended Complaint ("FAC") as to the first, second, third, fourth, and sixth causes of action for medical negligence, lack of informed consent, medical battery, fraud/concealment/deceit, and violation of Health and Safety Code section 123110.

RRSC also moves to

strike five categories of allegations: alter-ego or "acted as one" allegations, allegations imposing physician-level duties on RRSC, punitive-damages

allegations, medical-battery allegations, and allegations asserting a Health and Safety Code section 123110 violation based on record-production conduct.

Tentative Ruling

The demurrer is overruled as to the first, second, and third causes of action and sustained with leave to amend as to the fourth and sixth causes of action.

The motion to strike is granted as to punitive damages against RRSC. The motion to strike is otherwise denied. As to the section 123110 allegations, the

Court's ruling sustaining the sixth cause of action makes further striking unnecessary as moot.

Plaintiff is granted 10 days leave to amend to file a second amended complaint.

Demurrer: Legal Authority and Analysis

First Cause of Action: Medical Negligence

In a medical malpractice action, a plaintiff must establish the following elements: "(1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate causal connection between the negligent conduct and the resulting injury; and (4) actual loss or damage resulting from the professional's negligence. [citations.]" (Galvez v. Fields (2001) 88 Cal.App.4th 1410, 1420.)

In order for an organization to be found liable for medical negligence under a theory of ostensible agency, Plaintiff must prove two elements: "(1) conduct by the [organization] that would cause a reasonable person to believe that the physician was an agent of the [organization], and (2) reliance on that apparent agency relationship by the plaintiff." (Mejia v. Community Hospital of San Bernardino (2002) 99 Cal App. 4th 1448, 1454.)

The FAC alleges that the procedures at issue were performed at or about 125 N. Raymond Avenue, Suite 212, Pasadena, California 91103, and that each defendant was negligent in the selection, hiring, training, retention, and supervision of the others. (FAC, ¶¶ 4, 15.) The FAC further alleges that the procedures were performed through RRSC's facility by Dr. Tawansy, identified there as RRSC's sole managing member, using RRSC's facility, staff, equipment, scheduling process, consent procedures, operative records, and medical records. (FAC, ¶¶ 4, 27-28.)

As to duty, the FAC alleges RRSC was not merely the location of the surgeries but owned, operated, managed, controlled, staffed, maintained, scheduled, approved, and facilitated the surgical facility where plaintiff's procedures were performed, and that RRSC owed independent duties to ensure procedures were properly authorized, consent and consent documentation existed before surgery, prior operative records were reviewed before repeat procedures on the same patient and anatomical site, surgical protocols were followed, patient-safety policies were enforced, and unauthorized procedures were not permitted to occur at its facility. The FAC also alleges standard medical practice and reasonable surgical center protocols required review of prior surgical records before approving and facilitating a subsequent procedure on the same patient and anatomical site.

As to breach, RRSC argues the FAC targets only Dr. Tawansy's physician-level surgical decisions and pleads only generic facility allegations. But the FAC alleges more than a bare conclusion. It alleges RRSC, through its agents, staff, records systems, consent protocols, and facility procedures, participated in the surgical intake, consent process, scheduling, operative documentation, and post-operative documentation relating to plaintiff's procedures. It further alleges RRSC knew or should have known what procedures were being performed, what consent documents existed or did not exist, and whether further surgery was consistent with plaintiff's actual consent and the prior operative findings, yet nevertheless scheduled, approved, permitted, facilitated, and allowed the February 26, 2024 procedure to proceed without ensuring valid consent, without ensuring prior operative records were reviewed, and without preventing an unauthorized and unsafe repeat procedure. Although negligence may be pleaded generally, the complaint must still identify the acts or omissions claimed to be negligent.

As to causation, the

FAC alleges RRSC's conduct in scheduling, approving, permitting, facilitating, and allowing the February 26, 2024 procedure, and in failing to review prior operative records or enforce its protocols, was a substantial factor in causing plaintiff to undergo that procedure and in causing her resulting injuries. At the pleading stage, that is sufficient to allege proximate causal connection.

As to damages, the

FAC alleges the February 26, 2024 procedure caused permanent injury, including loss of vision. (FAC, ¶ 51.)

The FAC additionally

alleges Dr. Tawansy acted as RRSC's sole managing member, agent, managing representative, and authorized representative in connection with the procedures at RRSC's facility. These allegations satisfy the basic requirements for ostensible agency, which requires an organization, such as RRSC, to engage in conduct that would reasonably lead a plaintiff to believe that the relevant medical professional is employed by said organization.

For pleading

purposes, the FAC alleges duty, breach, causation, and damages against RRSC with sufficient specificity to state the first cause of action. The demurrer to the first cause of action is overruled.

Second Cause of Action: Lack of Informed Consent

"Because a patient

relies upon her physician's greater medical knowledge when seeking medical treatment, the physician has a fiduciary-like duty to obtain his patient's informed consent regarding which course of treatment to pursue." (Flores v. Liu (2021) 60 Cal.App.5th 278, 292 (Flores). Failure to do so constitutes negligence. (Ibid.)

This duty requires a physician "to

disclose to the patient all material information—that is, 'information which the physician knows or should know would be regarded as significant by a reasonable person in the patient's position when deciding to accept or reject a recommended medical procedure'—needed to make an informed decision regarding a proposed treatment." (Arato v. Avedon

(1993) 5 Cal.4th 1172, 1186.)

Material information

“include[s] (1) a ‘reasonable explanation of the [recommended] procedure[(s)],’ (2) the ‘likelihood of success’ of each recommended procedure, (3) ‘the risks involved in accepting [and] rejecting [each] proposed [procedure],’ particularly the ‘potential of death or serious harm’ and ‘the complications that might possibly occur,’ and (4) the physician’s ‘personal interests’ that may affect his judgment, even if ‘unrelated to the patient’s health.’” (Flores, *supra*, 60 Cal.App.5th at p. 293, quoting *Cobbs v. Grant* (1972) 8 Cal.3d 229, 243-245 (Cobbs).)

Because “the duty to obtain informed consent is pegged to what a ‘reasonable person’ in the patient’s position would deem to be ‘material’ to her medical decision-making (rather than being pegged to customs for disclosure in the profession), the decision as to what information should be disclosed is entrusted chiefly to the trier of fact, and not to medical experts.” (Id., at p. 294.) Other disclosures “might be material if ‘skilled practitioner[s] of good standing’ would ‘provide’ those disclosures ‘under similar circumstances.’” (Id., at p. 293, quoting *Cobbs*, *supra*, 8 Cal.3d at p. 244-245.) This does require the opinion of medical experts. (Id., at p. 294.)

The FAC alleges that

plaintiff’s consent was expressly limited to a “little by little,” phased course of surgeries on the right eye only, with each surgery intended to make only small, limited adjustments toward improving Plaintiff’s eye alignment and double vision. (FAC, ¶ 14.) The FAC alleges that plaintiff did not consent to any procedure on the left eye, and that defendants failed to inform plaintiff of the true facts concerning what procedures were performed and what consent was or was not obtained. (FAC, ¶¶ 14, 18.)

As discussed in the

first cause of action above, the FAC additionally alleges Dr. Tawansy acted as RRSC’s sole managing member, agent, managing representative, and authorized representative in connection with the procedures at RRSC’s facility. These allegations satisfy the basic requirements for ostensible agency.

At the demurrer

stage, the Court is not deciding the ultimate scope of any direct duty owed by RRSC. The Court concludes only that the

FAC adequately pleads a lack-of-consent theory against RRSC on the agency and facility allegations now pleaded. The demurrer to the second cause of action is overruled.

Third Cause of Action: Medical Battery

To establish a claim

for medical battery, a plaintiff must allege that (1) the defendant performed a medical procedure without consent or that plaintiff consented to one medical procedure, but performed a substantially different medical procedure, (2) that plaintiff was harmed, and (3) that conduct was a substantial factor in causing plaintiff's harm. (CACI No. 530A.) Further, "[w]here a doctor obtains consent of the patient to perform one type of treatment and subsequently performs a substantially different treatment for which consent was not obtained, there is a clear case of battery." (Cobbs v. Grant (1972) 8 Cal.3d 229, 239.)

The FAC alleges that

defendants performed procedures to which plaintiff did not consent, including procedures on a surgical site she did not authorize, and that plaintiff did not consent to any procedure on the left eye. (FAC, ¶ 17.) The FAC also alleges that the challenged procedures were carried out through RRSC's surgical facility using RRSC's staff, equipment, consent procedures, and records systems.

The demurrer's

categorical argument—that an entity cannot commit medical battery because it cannot physically touch a patient—is not supported by authority. California law recognizes medical battery where treatment is performed without consent or beyond the scope of consent. (Conte v. Girard Orthopaedic Surgeons Medical Group, Inc. (2003) 107 Cal.App.4th 1260, 1267 ["A typical medical battery case is where a patient has consented to a particular treatment, but the doctor performs a treatment that goes beyond the consent."].) The doctrine is not limited to individual defendants where the plaintiff also pleads agency, authorization, or institutional participation. (See, e.g., Stewart v. Superior Court (2017) 16 Cal.App.5th 87, 105-106 [the appellate court concluded that a hospital can be liable for medical battery when it authorizes an unconsented procedure].) At the pleading stage, the FAC alleges an unauthorized-procedure theory with enough specificity to proceed. The demurrer to the third cause

of action is overruled.

Fourth Cause of Action: Fraud, Concealment, and Deceit

“Under Civil Code section 1709, one is liable for fraudulent deceit if he deceives another with intent to induce him to alter his position to his injury or risk.... (Civ. Code, § 1709.)” (Beckwith v. Dahl (2012) 205 Cal.App.4th 1039, 1059–1060) [internal quotations omitted].) Deceit for the purposes of Civil Code section 1709 means “[a] promise, made without any intention of performing it. (Civ. Code, § 1710.)” (Beckwith, supra, 205 Cal.App.4th at p. 1060 [internal quotations omitted].) “The elements of fraud, which give rise to the tort action for deceit, are (a) misrepresentation (false representation, concealment, or nondisclosure); (b) knowledge of falsity (or scienter); (c) intent to defraud, i.e., to induce reliance; (d) justifiable reliance; and (e) resulting damage. [Citations.] [Citation.] Each element must be alleged with particularity.” (Beckwith, supra, 205 Cal.App.4th at p. 1060 [internal quotations omitted].)

RRSC’s demurrer

papers identify the FAC’s fraud theory as resting on alleged representations about a “little by little” surgical plan and on alleged concealment of the true nature of the procedures and records, but they also point out that the FAC attributes those statements to Dr. Tawansy and does not identify any RRSC employee or representative who spoke to plaintiff on RRSC’s behalf. RRSC’s reply likewise argues that the opposition still identifies only statements allegedly made by the surgeon and not any RRSC-specific communication.

The fraud claim is

not stated with adequate particularity as to RRSC. The demurrer to the fourth cause of action is sustained.

Sixth Cause of Action: Health and Safety Code Section 123110

“[A]ny adult patient

of a health care provider . . . and any patient’s personal representative shall be entitled to inspect patient records upon presenting to the health care provider a request for those records and upon payment of reasonable costs.” (Health & Safety Code, § 123110(a).)

“A health care

provider shall permit this inspection during business hours within five working days after receipt of this request.” (Id.)

“Any patient or her representative shall

be entitled to a paper or electronic copy of all or any portion of her records that they have a right to inspect, upon presenting to the healthcare provider a request specifying the records to be copied.” (Health & Saf. Code § 123110(b)(1).) “A health care provider shall ensure that copies are transmitted within 15 days after receiving the request.” (Health & Safety Code, § 123110(b)(1).)

“Any patient or representative aggrieved by a violation of Section 123110 may, in addition to any other remedy provided by law, bring an action against the health care provider to enforce the obligations prescribed by Section 123110. Any judgment rendered in the action may, in the discretion of the court, include an award of costs and reasonable attorney fees to the prevailing party.” (Health & Safety Code, § 123120.)

The FAC alleges that plaintiff made multiple written requests for complete medical records on or about February 20, 2025 and thereafter, including requests for operative reports, surgical records, consent documents, and related records, and that defendants failed to produce them or produced only blank or template forms. (FAC, ¶ 20-22.) The FAC further alleges that defendants’ continued refusal to comply with those requests violates Health and Safety Code section 123110 and seeks statutory damages and penalties. (FAC, ¶ 92.)

RRSC argues that the FAC fails to allege a proper statutory request to RRSC, fails to specify RRSC’s noncompliance, and seeks relief beyond that authorized by statute. (Demurrer, p. 16.) As pleaded, the sixth cause of action does not clearly separate a statutory patient-records request from later discovery disputes and does not clearly plead the RRSC-specific request, noncompliance, and remedy theory on which the claim depends. The demurrer to the sixth cause of action is sustained.

Motion to Strike: Analysis

RRSC’s motion to strike seeks to remove alter-ego and “acted as one” allegations, physician-level duty allegations, punitive-damages allegations, medical-battery allegations, and section 123110-related allegations.

The FAC prayer seeks punitive damages against Dr. Tawansy and the corporate defendants for alleged fraudulent conduct and reckless disregard for plaintiff’s safety.

Because the fraud claim against RRSC is not adequately pled, the motion to strike is granted as to the punitive-damages request against RRSC.

The motion to strike is otherwise denied. The challenged alter-ego, agency, facility-conduct, consent, and battery allegations overlap with claims the Court is allowing to proceed.

As to the section

123110 allegations, the Court's ruling sustaining the sixth cause of action makes further striking unnecessary as moot.

Moving Party is to give notice.